



Guideline

Leave Management in Community Older Adult Mental Health Service

Scope (Staff): All NMHS Older Adult Community Mental Health Service Staff

Scope (Area): All NMHS Older Adult Community Mental Health Service Areas

Contents

1 Aim	1
2 Background	1
3 Risk	2
4 Key Points	2
5 Maximum Leave Allocation.....	3
6 Procedure for Applying for Leave	3
6.1 Annual Leave & Accrued Days Off (ADO)	3
6.2 Peak Periods	4
6.3 Long Service Leave	5
6.4 Personal Leave	5
6.4 Professional Development Leave.....	5
6.5 COVID Leave	6
6.6 Purchased Leave	6
6.7 Parental Leave	6
7 Application to Cash Out Leave Entitlements	6
8 Right of Appeal.....	7

1 Aim

This guideline outlines leave management principles, responsibilities and strategies within the Older Adult Community Mental Health Service (OACMHS) for leave to be processed in a fair, timely, and efficient manner to meet the needs of both the individual and the service.

2 Background

The scheduling and processing of leave applications for staff members is an organisational requirement.



The Business Rules ensure:

- Staff members receive leave entitlements appropriate under the various industrial awards in an efficient and timely manner; and
- Leave is scheduled in a fair and equitable manner ensuring efficient and effective functioning of the service in meeting the needs of our clients.

To assist this process, the OACMHS has established guidelines to:

- identify adherence requirements of staff when applying for the various categories of leave;
- identify the approval process in accordance with the NMHS Leave Management Policy; and
- ensure maximum opportunities are provided to clear leave on a yearly basis before it becomes excess leave.

3 Risk

Non-compliance with this guideline may result in of inequitable allocation of leave and excess leave accruals, in addition to impact on:

Culture	☒	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☐
Fiscal Responsibility	☒	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

4 Key Points

- Leave applications will be considered in relation to operational requirements which include the availability of suitable cover (if required), cost implications, impact on client service requirements and the impact to other employees.
- It is the responsibility of each staff member to check the team leave planner and obtain preliminary approval from their line manager prior to submitting leave applications, to ensure their application falls within the maximum leave allocations (see below).
- It is the responsibility of each staff member to ensure leave is submitted electronically using an HSS leave form to their line manager. The leave form should be accessed from the HSS website; stored copies of forms may not be the current version in use.
- Leave forms will list one period of continuous leave only.
- Senior staff who approve leave applications are responsible for ensuring adherence to minimum numbers (as below). Consideration should be given to the psychological wellbeing and fatigue management of remaining staff.



- Leave applications should be approved or declined within two weeks of being sent to the line manager.
- Every effort will be made to backfill periods of leave of more than two weeks. Suitable notice needs to be given to the line manager for backfill to be arranged.
- Consideration will be given to the provision of higher duties in maintaining team seniors.
- For all periods of planned absence, employees should ensure their caseload has been handed over as per the Clinical Handover Policy and OAMHS Clinical Capacity Management procedure.
- Employees are required to ensure an 'out of office' reply is set on their Outlook prior to leaving the workplace for planned leave; the message should stipulate the return to work date, and an alternate contact during their period of leave. This applies for any period of leave, including single days.
- There are rare occasions when operational needs will require cancellation of booked leave. This will be in exceptional circumstances and will be decided in conjunction with the line manager, Service Manager and/or Coordinator of Nursing.
- It should be noted that, while every staff member has leave entitlements under their specific award, the taking of leave is an agreement between the employer and the employee. Where excess leave has accumulated, the employee will be encouraged to apply for leave in consultation with their line manager. Any requirement for an excess leave management plan will be notified to the employee as appropriate.
- Staff may contact HSS directly on 13 44 77 for details regarding leave entitlements, and view information online at Your Leave Entitlements (through HealthPoint Online)

5 Maximum Leave Allocation

Leave application and approvals within community teams should not leave the team with less than 50% of the normal complement of staff. This number includes staff on Worker's Compensation leave and is in compliance with OAMHS Clinical Capacity Management procedure.

6 Procedure for Applying for Leave

6.1 Annual Leave & Accrued Days Off (ADO)

The following procedures for leave applications and ADOs will apply:

- Discuss leave requests with your supervisor using the leave planner as a guide.
- Submit your application to your line manager accessing the leave form via the HSS website (e-forms and paper forms will not be accepted). The line manager should not support leave request unless there is evidence that the application has been discussed and supported with the clinical / administrative team and is within the staffing limitations stated above.



- The line manager should note endorsement in the comments box of the form and forward the form to HSS, copying in the onsite RoStar manager, and will save the approved form in the appropriate folder within the W drive.
- It is preferred that annual leave applications are submitted six weeks in advance where possible, and no less than two weeks prior to requested leave, unless in exceptional circumstances - eg. bereavement or serious illness of a close relative.
- Requests for ADOs or other single leave days should give at least one week's notice unless otherwise agreed with line manager, or in exceptional circumstances - eg. bereavement or serious illness of a close relative.
- ADO or personal leave of less than one week should be entered directly into RoStar by the RoStar Manager, and the approved leave form will be saved in the appropriate folder within the W drive.
- Any type of leave of less than one whole day will be supported with an approved leave form and entered directly into RoStar by the RoStar Manager; the form is not to be sent to HSS, but will be saved in the appropriate folder within the W drive.
- Any type of approved leave requests that falls within the current or previous fortnight's RoStar period will be entered directly into RoStar by the RoStar Manager, and the approved leave form will be saved in the appropriate folder within the W drive.
- Leave discussions that take place within Business Meetings or clinical team meetings will be recorded in the minutes of the Team Business Meeting.
- **Employees should be mindful that travel arrangements and accommodation bookings should not be confirmed until the leave request is approved.**

6.2 Peak Periods

This period includes school holidays, Easter, Christmas and New Year. Peak periods are not classified within the Service as low activity periods.

- Any decision to grant leave is **not** based on 'first come, first served'.
- In the event of multiple staff requests for leave over the same period, the team will discuss and reach a provisional agreement. Should the team be unable to reach agreement, the line manager with the assistance of senior staff as required will arbitrate until a final decision is reached.
- The decision to grant leave will be made by taking into consideration the needs of the service, the needs of the employee, and any previous history of taking leave during peak periods.
- This is to ensure that every staff member has an equitable opportunity to take leave during a peak period.



6.3 Long Service Leave

The application and approval procedure for Long Service Leave will follow the same practice as the process for Annual Leave.

Staff should check their Award and ensure that they are submitting applications within the required timeframe.

6.4 Personal Leave

This includes sick leave, carer's leave and bereavement leave.

All industrial Awards specify the responsibilities of employees when taking sick leave. Please see your Award and ensure that you abide by the conditions set out regarding certification of sick leave.

Please adhere to the relevant local procedure when requesting and notifying these leave types, noting the following:

- Staff should, where practicable, speak directly to their line manager to report their absence. This includes when a staff member needs to leave work early due to illness and/or appointment.
- Upon returning to work, the employee will complete the leave form and submit to their line manager on the same day of recommencing duty. Staff should submit medical certificates for leave longer than two days with their application where applicable (eg. sick leave, carer's leave).
- The line manager should reconcile the employee's application form to ensure that the correct leave category and number of days/hours are recorded.
- The approved leave form will be forwarded to HSS (as appropriate) and saved in the appropriate folder within the W drive; the leave will be entered into RoStar by the RoStar manager.
- Planned personal leave dates should be communicated to the team and appropriate handover provided, where required.

6.4 Professional Development Leave

To monitor staffing numbers in each team, professional development leave will be recorded in RoStar.

- **Approval is required for all courses where professional development leave is requested.**
- Staff are expected to check the Leave calendar to ascertain whether staffing levels support the leave request prior to requesting leave, and encouraged to discuss the application with their line manager.



- A study leave request form and pertinent course information is to be provided for all courses except:
 - Core Competencies - eg. mandatory training (run by Dept of Health)
 - Any training directed or recommended by your line manager run by Dept of Health
- All approved study leave forms and accompanying completed and authorised leave forms will be tracked and stored in the appropriate folder on the W drive.
- Professional development leave will be entered onto RoStar by the RoStar Manager, using the appropriate code (PC **not** ST) for the staff member.
- Professional development leave will only be considered for use on an employee's rostered days unless the leave is for training considered as mandatory training or a requirement under the employee's job description (JDF). In this case there may be an option to swap a day, use any accrued TOIL (if applicable to the site) or be paid the additional hours.

6.5 COVID Leave

Refer to the MHPHDS Hub for the latest COVID-19 policies, procedures and entitlements.

6.6 Purchased Leave

Staff may apply for purchased leave as per their appropriate Award who will submit the application, along with supporting documentation, to the Service Manager / Coordinator of Nursing for approval.

Requests will be determined on a case by case basis, endeavoring to ensure equity of allocation of purchased leave based on operational needs, personal leave accrual/balance and demand.

6.7 Parental Leave

The employee will provide a minimum of ten (10) weeks of notice in advance of the expected date of birth. Medical evidence and written notification of the proposed commencement of Parental Leave to be taken is required.

The approved leave form will be forwarded to HSS, and saved in the appropriate folder within the W drive

The leave will be generated automatically on RoStar, but should be monitored by the RoStar Manager for the end date and discussions with the employee regarding a return to work should commence.

7 Application to Cash Out Leave Entitlements

An Application to Cash Out Leave Entitlements form (L13) may be submitted to the line manager, for consideration and discussion with the Service Manager / Coordinator of Nursing as appropriate, prior to any submission to the Co-Director. Applications should include a rationale for the request.



8 Right of Appeal

If any staff member is concerned about a decision made in relation to this leave policy, they are encouraged to discuss with their line manager in the first instance as per the WA Health Grievance Policy.

Related internal policies, procedures and guidelines

NMHS Leave Management Policy: [NMHS Leave Management Policy](#)

NMHS Employee Leave Management Plan: [Employee Leave Management Plan \(NMHS\)](#)

Clinical Handover Policy: [WA Department of Health Clinical Handover Policy](#)

Clinical Capacity Management Procedure: [Clinical Capacity Management Procedure \(OAMHS\)](#)

Policy Sponsor	Clinical Nurse Manager - Community				
Policy Contact	Clinical Nurse Manager – Community (Wanneroo Older Adult)				
Date First Issued:	19/03/25	Last Reviewed:		Review Date:	19/03/2028
Approved by:	MH Policy Coordination Committee			Date:	19/03/2025
Endorsed by:	MH Policy Coordination Committee			Date:	19/03/2025
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance				
National Standards for Mental Health Services	☒ Std 2: Safety ☒ Std 8: Governance, leadership, and management				
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning,				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Committee Authorising Document	Date Authorised	Minutes Ref No	Document Reference
Senior Management Sub Committee	06/02/25	12.2	Minutes February 2025

Document Version Control			
Date	Version	Author	Notes
03/2025	1	CNM - Wanneroo	First Issue

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2024

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.

